MACQUARIE UNIVERSITY Health
MACQUARIE MEDICAL IMAGING

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Patient Name:	WANG, DONGPING	**Accession Number:**	3312736-CT
Patient ID:	MQ152436	**Examination Date:**	April 1, 2026 12:56
Gender:	Male	**Report Status:**	Final
Date of Birth:	January 7, 1957	**Modality:**	CT
Referring Physician:	YAN, TRISTAN		

Report for Computed Tomography

CT THORACIC AORTIC ANGIOGRAM

CLINICAL HISTORY: Yearly review Post AVR, ascending aortic replacement and hemiarch replacement.

Comparison made with previous CT the aorta from 29/10/2024.

TECHNIQUE:

MD CT scans were obtained through the thoracic aorta following the angiogram protocol. The aorta was analysed on the GE volumeshare workstation and measurements obtained.

REPORT:

Aortic arch:

Previous aortic valve replacement, ascending aorta and hemi arch replacement is noted.

Location	Previous size	Current size
Aortic annulus	22 x 29mm	19 x 30mm
Sinus of Valsalva	38 x 43mm	39 x 40mm
Sinotubular junction	39 x 41mm	36 x 40mm
Mid ascending aorta	48 x 50mm	29 x 32mm
Distal ascending aorta	44 x 46mm	33 x 39mm
Mid arch (between L CC and left subclavian)	35 x 37mm	36 x 39mm
Distal arch	28 x 33mm	29 x 33mm
Proximal descending aorta	28 x 29mm	28 x 30mm

Aortic Arch Branches:

Innominate: It has conventional anatomy & no stenosis is seen.

Left common carotid: There is no significant atheroma at its origin or stenosis.

Left subclavian: There is minor calcified atheroma at its origin with less than 25% stenosis.

Descending aorta:

There is scattered calcified atheromatous disease is present.

Heart:

The coronary arteries have conventional origins and there is scattered coronary artery calcification.

The heart is not enlarged. No pericardial effusion is seen.

Mediastinum:

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There is no mediastinal or hilar lymphadenopathy. The solid nodule abutting the lower oesophagus in the posterior mediastinum is again noted measuring 20 x 17 mm (previously 22 x 18 mm on the MR from 10/4/2025.)

Since the previous study, there has been interval development of extensive venous collaterals throughout the thorax, raising the possibility of a central venous stenosis at the level of the innominate vein.

Lungs & Pleural spaces:

No focal consolidation is seen. There is a stable 6 mm left upper lobe pulmonary nodule. No other suspicious pulmonary nodules or mass.

No emphysematous change is present. No pleural fluid collection is seen and there are no pleural plaques.

Supra-clavicular region & axilla:

No lymphadenopathy is seen. The thyroid is not enlarged.

Other:

No rib lesion or fracture is identified. No destructive vertebral body lesion is seen.

CONCLUSION:

Interval ascending aorta and hemiarch replacement.

Development of new extensive venous thoracic collaterals, likely due to innominate vein stenosis.

Stable appearance of the posterior mediastinal nodule abutting the lower oesophagus and the left upper lobe nodule.

Radiologist: Dr D. Ravindran